ENGLISH FOR WORK / SEPTEMBER 2026

Medical Devices English

Vocabulary & phrasebook

Keep precise meanings and useful expressions close. Retrieve the words, then use them in a complete message.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Find the right language. Speak, write, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For medical device engineers, product managers, quality and regulatory staff, clinical specialists, manufacturing teams, field-service teams, and device-company leaders.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

Open this course on English Ladder

Make vocabulary usable

Knowing a definition is a start. To use a term well, explain it in plain English, connect it to a case, and choose a natural sentence around it.

- Read five terms, cover their definitions, and recall the meanings.

- Sort a pair you might confuse and explain the difference.

- Use one term in a sentence about a fictional case.

- Ask a partner to explain the sentence without repeating the specialist word.

- Return to the same terms on another day and test recall again.

Abbreviations need context

Expand an abbreviation on first use when the listener needs it. Some abbreviations have different meanings across professions: API can refer to an application programming interface or an active pharmaceutical ingredient. The course context determines the meaning.

Definitions have limits

These are concise language-learning explanations, not complete legal, clinical, or technical specifications. Use the applicable authoritative definition for regulated or operational decisions.

Field vocabulary A-Z

510(k)
US FDA premarket submission that typically demonstrates substantial equivalence to a legally marketed predicate device.

acceptance criteria
Specific conditions a deliverable must meet to be accepted.

clinical evidence
Information from clinical evaluation or research relevant to a health-related question.

complaint
An expression of dissatisfaction requiring attention under a relevant process.

De Novo
A US FDA classification process for certain novel devices without a legally marketed predicate.

design input
A documented requirement that guides a product's design.

design review
A structured evaluation of a design at a defined stage.

FMEA
Failure mode and effects analysis, a structured method for identifying how a process or product can fail and prioritizing controls.

formative study
An evaluation used during development to identify problems and improve a design.

harm
Injury, damage, or another adverse effect on people, property, or the environment.

hazard
A potential source of harm.

human factors
The study and application of knowledge about how people interact with products, tasks, and environments.

instructions for use
The manufacturer's information explaining the intended use and operation of a device.

intended use
The purpose for which a product is designed and represented to be used.

labeling
The information supplied with or about a regulated product, as defined by applicable rules.

lot traceability
Ability to follow a production lot through materials, process steps, tools, inspections, and downstream customers or products.

malfunction
A failure of a product or system to operate as intended.

MDR
Medical Device Reporting: the US FDA system for specified device-event reports; elsewhere the acronym may refer to a regulation.

nonconformance
Failure to meet a requirement, specification, contract, standard, or approved procedure.

off-label
A use outside a regulated product's approved or cleared labeling, as applicable to that product and jurisdiction.

PMA
Premarket approval: the US FDA review pathway for certain high-risk medical devices.

postmarket surveillance
Collecting and evaluating information about a product after it enters the market.

predicate device
A legally marketed device used as a comparison in an applicable US FDA submission.

process validation
Documented evidence that a process can consistently produce results meeting specified requirements.

residual risk
The risk remaining after controls or other responses are applied.

summative study
An evaluation conducted to assess performance against defined requirements after development work.

supplier CAPA

A supplier's corrective and preventive action process for addressing identified quality problems and recurrence risks.

traceability

The ability to follow an item's history, source, requirements, or records through documented links.

use error

An action or omission during use that leads to a different result from the intended one; context and design matter.

user need

Something a user needs to accomplish in a particular context.

validation

Confirming that an output or system meets needs for its intended use under defined conditions.

verification

Checking whether specified requirements have been met.

Expressions for this course

Complete the frames with the facts of your case. A frame helps organize meaning; it should not replace an actual explanation.

Ask a precise question

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is ... . Is that right?

Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

Make a complex point clear

In this context, ... means ... .

The difference is that ... .

How would you explain that distinction to a colleague?

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

Explain a fair comparison

Both options ..., but ... .

Option A ..., whereas option B ... .

The comparison depends on ... .

Use the same time period, scope, and measurement basis for both options. Explain a difference with 'whereas' or 'while'. State what improves and what is given up; do not hide the weaker side of a recommendation.

Match certainty to evidence

The available evidence shows ... .

This may indicate ..., but ... .

We have not yet established whether ... .

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

Ask for an actionable next step

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know ... .

State the action and the information needed. Explain why it matters and specify a deadline only when one is given or agreed. 'Could you' is polite; repeated apologies can obscure a legitimate request. Urgent situations may require a direct request.

Transfer information and responsibility

The current status is ...; the outstanding item is ... .

The record shows ... .

Please confirm who has accepted ... .

State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

Give a useful status update

We have completed ...; ... remains open.

As of ..., the status is ... .

The next update will cover ... .

Lead with the current state, then the important change or open item. Use completed-action language only for completed work. Add a next update point when it is known; do not invent a date to make the message sound complete.

Disagree with a useful reason

I understand the goal. My concern is ... .

That statement goes beyond ... .

Could we ... before confirming ...?

Acknowledge the goal without pretending agreement. Name the specific problem and its implication, then ask a focused question or propose a next step. Be direct when a safety or ethical boundary requires it.

Model responses in context

Read the situation first. Cover the model, say your own response, and compare the two for meaning and tone.

01 | User Needs and Design Inputs

A design brief says the device should be "easy to use." It does not identify the intended users or use environment.

Who will use it, for which task, and in what environment? Let's turn "easy to use" into specific user needs and requirements that the team can review.

Try it again: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

02 | Risk Management and Hazard Analysis

A risk discussion uses hazard and harm interchangeably. The team needs to describe the potential source of harm and the possible outcome separately.

Let's distinguish the hazard from the harm that could result. That will help us describe the sequence clearly and discuss the controls without treating the two terms as synonyms.

Try it again: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

03 | Verification, Validation, and Design Review

A device team has checked design outputs against specifications but has not yet completed the planned evaluation of intended user needs.

The specification checks address verification. They do not by themselves answer every validation question about intended use. I'll report those activities separately and identify what remains open.

Try it again: Your listener focuses only on the lowest price or fastest date. Explain one other relevant comparison criterion.

04 | Usability and Human Factors

Several participants misread a label in a formative usability study. A draft report describes them as careless.

The observed difficulty was reading the label correctly in this study. Let's describe the task, design, and use conditions, then investigate contributing factors without assuming carelessness.

Try it again: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

05 | Regulatory Pathways and Submissions

A presentation refers to a proposed device pathway as settled, although the regulatory team is still evaluating options.

Please label the pathway as under evaluation and identify the questions for the regulatory team. We should distinguish a proposed approach from a confirmed regulatory conclusion.

Try it again: The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

06 | Complaints, MDRs, and Postmarket Signals

A complaint report describes a malfunction but lacks the device identifier and event date. The complaint-handling team needs the available facts promptly.

Here is the reported malfunction and the information currently available. The device identifier and event date are missing. Please confirm receipt and handle follow-up through the complaint process.

Try it again: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

07 | Manufacturing, Suppliers, and Nonconformance

An incoming-material review has identified two nonconforming lots. The investigation is open and disposition has not been authorized.

Two lots have open nonconformance records. The investigation and disposition are still pending. I'll report their status without implying that release or rejection has already been approved.

Try it again: The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

08 | Clinical Training and Labeling Boundaries

A trainer is asked to demonstrate a use beyond the materials approved for the session.

That request goes beyond the approved training material for this session. I can cover the authorized instructions and route the additional question to the appropriate team for review.

Try it again: The other person repeats the request more firmly. Restate the specific concern calmly and explain the appropriate next route.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

https://digital.gov/guides/plain-language

Council of Europe: CEFR descriptors

https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors

FDA: De Novo classification

https://www.fda.gov/medical-devices/premarket-submissions-selecting-and-preparing-correct-submission/de-novo-classification-request

FDA: Medical Device Reporting

https://www.fda.gov/medical-devices/medical-device-safety/medical-device-reporting-mdr-how-report-medical-device-problems

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.